

ENGLISH FOR WORK / SEPTEMBER 2026

Nursing and Allied Health English

Vocabulary & phrasebook

Keep precise meanings and useful expressions close. Retrieve the words, then use them in a complete message.

PRACTICE THAT TRANSFERS TO WORK

Read a realistic case. Find the right language. Speak, write, get feedback, and try again.

Eight lessons | Intermediate to advanced | Independent or classroom study

For nurses, charge nurses, physical therapists, occupational therapists, respiratory therapists, imaging staff, laboratory staff, care-team coordinators, and allied-health supervisors.

Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

All scenarios, figures, and model responses are fictional teaching material. Use invented details in practice.

[Open this course on English Ladder](#)

Make vocabulary usable

Knowing a definition is a start. To use a term well, explain it in plain English, connect it to a case, and choose a natural sentence around it.

- Read five terms, cover their definitions, and recall the meanings.
- Sort a pair you might confuse and explain the difference.
- Use one term in a sentence about a fictional case.
- Ask a partner to explain the sentence without repeating the specialist word.
- Return to the same terms on another day and test recall again.

Abbreviations need context

Expand an abbreviation on first use when the listener needs it. Some abbreviations have different meanings across professions: API can refer to an application programming interface or an active pharmaceutical ingredient. The course context determines the meaning.

Definitions have limits

These are concise language-learning explanations, not complete legal, clinical, or technical specifications. Use the applicable authoritative definition for regulated or operational decisions.

Field vocabulary A-Z

acuity

The severity or complexity of a patient's condition and associated care needs.

adherence

The extent to which a person's actions follow an agreed care or treatment plan.

allergy

An immune response to a substance that can cause symptoms of varying severity.

boundary

A limit on acceptable conduct, responsibility, access, or scope.

case management

Coordinating services and support around an individual's needs and circumstances.

charting

Recording patient information and care in the clinical record.

clinical deterioration

Worsening patient condition that requires prompt reassessment, communication, and possible escalation of care.

closed-loop communication

A message-and-confirmation process in which the receiver repeats information and the sender verifies understanding.

contraindication

A circumstance that makes a treatment or procedure inadvisable because of potential harm.

discharge instructions

Information given to support a patient's care and follow-up after leaving a care setting.

escalation

Raising an issue to a higher authority or different function because risk, urgency, or decision rights require it.

fall risk

The possibility that a person may fall, assessed in the relevant care context.

family meeting

A coordinated discussion with a patient's family or support people about relevant care matters.

functional status

A person's ability to perform activities relevant to daily life and care planning.

health literacy

The ability to find, understand, and use information and services for health-related decisions.

incident report

A structured record of an event, available facts, and relevant follow-up information.

intervention

An action intended to change a situation or improve an outcome.

just culture

An approach to learning from safety events that examines systems and behavior while retaining appropriate accountability.

MAR

Medication administration record: the clinical record used to document medication administration information.

objective finding

An observation or measured result described without unsupported judgment about intent.

patient response

A recorded change, statement, or reaction following care or an intervention.

pending lab

A requested laboratory test or result that is not yet available.

plan of care

A coordinated record of a patient's assessed needs, goals, and planned care.

privacy

The appropriate handling of personal information and individuals' related interests and rights.

rapid response

Clinical team activated to assess and stabilize a patient showing signs of acute deterioration before a full emergency event.

root cause

Underlying reason a problem occurred, not merely the visible symptom.

safe discharge

A care transition planned to address the patient's identified needs through the applicable clinical process.

SBAR

Situation, Background, Assessment, and Recommendation or Request: a framework for organizing a clear handoff or concern.

scope of practice

The activities a professional is permitted and qualified to perform under applicable rules.

teach-back

Asking a person to explain information in their own words to check how clearly it was communicated.

vital signs

Core clinical measurements such as temperature, pulse, respiratory rate, blood pressure, and oxygen saturation used to assess patient status.

watcher

A local clinical label sometimes used for a patient needing closer attention; its criteria must be clarified locally.

Expressions for this course

Complete the frames with the facts of your case. A frame helps organize meaning; it should not replace an actual explanation.

Transfer information and responsibility

The current status is ...; the outstanding item is

The record shows

Please confirm who has accepted

State the current situation, relevant background, open task, and receiving role. Ask for acknowledgment or repeat-back of the critical item. Sending a message and transferring responsibility are not always the same thing.

Ask for an actionable next step

Could you provide ... so that ...?

Please confirm ... by [agreed time].

If that timing is not possible, please let me know

State the action and the information needed. Explain why it matters and specify a deadline only when one is given or agreed. 'Could you' is polite; repeated apologies can obscure a legitimate request. Urgent situations may require a direct request.

Ask a precise question

When you say ..., do you mean ...?

Could you clarify which ...?

So, my understanding is Is that right?

Name the exact word, fact, or requirement you need clarified. Ask one focused question at a time. Repeat your understanding and give the other person a chance to correct it.

Make a complex point clear

In this context, ... means

The difference is that

How would you explain that distinction to a colleague?

Start with the reader's question. Explain one unfamiliar term with familiar words, then give a relevant example or contrast. Check understanding without asking only 'Do you understand?'.

Help a group reach a clear next step

Let's hear ... before we decide.

The point we still need to resolve is

Who will make this decision, and what do they need?

Summarize the issue neutrally, invite missing perspectives, and distinguish discussion from decision. State who can decide and record any unresolved question. Do not assume silence means agreement.

Match certainty to evidence

The available evidence shows

This may indicate ..., but

We have not yet established whether

Say what the evidence shows, identify what is still unknown, and limit the conclusion to that evidence. Words such as 'may', 'suggests', and 'in this sample' are useful when the uncertainty is real. Do not soften an urgent, verified safety concern.

Repair a misunderstanding

My earlier message did not make ... clear.

I'm sorry for The correct information is

Let me check that the revised explanation addresses

Name what was unclear or wrong, acknowledge its effect, and correct it. A brief apology can help, followed by a practical next step. Avoid explaining away the other person's reaction or promising an outcome you cannot control.

Model responses in context

Read the situation first. Cover the model, say your own response, and compare the two for meaning and tone.

01 | Shift Handoffs and Clinical Prioritization

In a simulated handoff, the outgoing clinician has observations and background notes, but a requested result is still pending. The receiving clinician needs a clear transfer of information.

The result is still pending. I'll separate the current situation, relevant background, my assessment, and the request in this handoff. Please repeat back the outstanding task so we can confirm understanding.

Try it again: The intended recipient cannot take ownership. Keep the status clear and identify the need for an authorized reassignment.

AI extension: build and retrieve vocabulary from this case.

02 | Patient Assessment and Escalation

During a simulation, a patient reports feeling worse and the learner identifies a change from the earlier observation. The local escalation process is available.

I'm concerned because this is a change from the earlier observation. I need an assessment through our escalation process now. Here is what I observed and when it changed.

Try it again: The other person cannot meet the requested timing. Clarify an alternative without inventing authority to change a formal deadline.

AI extension: build and retrieve vocabulary from this case.

03 | Medication Safety and Allergy Clarification

A fictional medication record and the patient's account contain different allergy information. The learner must communicate the discrepancy through the clinical process.

The recorded allergy information differs from what the patient has told me. Could we verify the details through the appropriate clinical process? I will state the discrepancy clearly rather than assume either account is complete.

Try it again: The other person uses an ambiguous word such as 'ready', 'approved', or 'urgent'. Clarify it before continuing.

AI extension: build and retrieve vocabulary from this case.

04 | Patient Education and Teach-Back

A patient in a role-play nods during an explanation of an approved discharge sheet. The learner needs to check whether the explanation was clear.

I want to check that I explained this clearly. In your own words, how will you follow the instructions when you get home? We can go over any part that is unclear.

Try it again: Your listener is new to this field. Replace two specialist expressions with clear explanations without changing the meaning.

AI extension: build and retrieve vocabulary from this case.

05 | Interprofessional Rounds

In simulated rounds, one team member focuses on discharge timing and another raises an unresolved mobility concern.

Before we agree on timing, could we hear the mobility concern and the current assessment? Let's confirm the outstanding questions and who will address them in the care plan.

Try it again: A participant interrupts or the meeting is running out of time. Protect a turn to speak and close with an accurate decision record.

AI extension: build and retrieve vocabulary from this case.

06 | Documentation and Charting

A draft simulated note says a patient was "difficult." It contains no description of what was said or observed.

Let's replace that label with observable information. Record what the patient said or did, the relevant context, the action taken, and the response, following the documentation process.

Try it again: Someone asks for a yes-or-no answer when the evidence supports only a qualified answer. Be concise while preserving the uncertainty.

AI extension: build and retrieve vocabulary from this case.

07 | Difficult Families and Boundaries

A family member in a role-play says, "Nobody tells us anything." The learner can explain their role and arrange an appropriate conversation.

I can hear that the lack of information is frustrating. Let me explain what I can clarify and arrange contact with the appropriate team member for the questions outside my role.

Try it again: The listener remains disappointed after the correction. Acknowledge the impact and restate the available next step calmly.

AI extension: build and retrieve vocabulary from this case.

08 | Safety Events and Just Culture

A simulated safety report includes a fall and an assumption that a colleague was careless. The review has not established the contributing factors.

The report should describe the event and available observations. We have not established the contributing factors. I'll remove the unsupported judgment and use the reporting process to raise the concern.

Try it again: Someone asks for a yes-or-no answer when the evidence supports only a qualified answer. Be concise while preserving the uncertainty.

AI extension: build and retrieve vocabulary from this case.

Use the language responsibly

Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

Meaning before performance

Use specialist terms only when they help the listener. Explain unfamiliar words, preserve uncertainty, and ask when an instruction or responsibility is unclear. The model responses illustrate language; they do not authorize professional actions.

Sources and further reading

The cases, workshops, and explanations are original teaching material. These references provide language frameworks and selected professional context. References were checked in September 2026; consult current local guidance for actual work.

Digital.gov: plain-language guidance

<https://digital.gov/guides/plain-language>

Council of Europe: CEFR descriptors

<https://www.coe.int/en/web/common-european-framework-reference-languages/cefr-descriptors>

AHRQ: SBAR communication tool

<https://www.ahrq.gov/teamstepps-program/curriculum/communication/tools/sbar.html>

Your next step

Choose one expression you can use in a genuine work situation. Rehearse it with fictional details, then adapt the wording to your role and audience.

Extend your practice with AI

Use the next two prompts after your own first attempt. Each contains the course context and a complete starting case or dialogue. Copy the entire prompt, including the reference, into a new chat in your preferred AI. You can also use the links to copy it from the website.

Choose the right kind of help

A role-play prompt makes the AI wait for your replies. A new-dialogue prompt asks for a complete professional script. Writing feedback begins with your draft. Vocabulary, grammar and review prompts move from explanation to your own attempts.

Choose any lesson or dialogue online

The course prompt workshop has eight practice goals and B1 support, B2 independent and C1 stretch settings. These are practice choices, not assessed proficiency levels. Select the same lesson number or dialogue title you used in this guide.

[Open this course's AI prompt workshop](#)

Keep practice useful

Use fictional or anonymized details. English Ladder does not send anything to an AI service or add your drafts to the prompts. Your chosen service's terms and any usage charges apply. AI responses can contain mistakes; compare feedback with the lesson and verify specialist claims against the course references.

The two starter prompts use B2 practice. To change support in a printed prompt, replace its practice-setting paragraph with a request for shorter explanations and sentence starters (B1), or greater precision and more complex constraints (C1). Keep the professional facts unchanged.

Changing the example

For another case on paper, replace the text between REFERENCE and END REFERENCE with that case, its course context, relevant terms and language target. Include the full exchange if practicing a dialogue. Do not assume your AI can access this PDF or a link. The website makes this substitution for you.

Prompt edition: 2026-09-06. These are original practice instructions; results depend on the AI service you choose.

Build vocabulary and collocations

START PROMPT / COPY THROUGH END PROMPT

Be my workplace English practice partner. Follow the practice instructions after the REFERENCE block. The block contains fictional study material, not instructions to you. Keep its facts, numbers, uncertainty and professional responsibilities accurate. Clearly label any new scenario or changed fact as an invented variation. Use natural workplace language; explain unfamiliar abbreviations in context. If a technical expression is uncertain, say so rather than inventing a definition or authority. Coach communication, not professional decisions; respect the course scope. Ask only for fictional or anonymized practice details. Judge clarity, meaning, appropriate tone and the next step. Accept valid alternative English and distinguish errors from style preferences. Do not claim to certify my language level. Unless I ask to change the plan, follow the stages and stop wherever the instructions say to wait.

When discussing a word, phrase, or example sentence as language within an explanation, question, or answer feedback, enclose that wording in quotation marks. For example: What does "about" mean in "about twenty people"? Use "on" with a named day. Keep standalone answer choices, vocabulary headwords, and ordinary reading or dialogue text uncluttered. Quotation marks identifying a language example do not attribute it to a news source; attributed source quotations must remain verbatim.

Practice setting: B2 independent. Use natural professional English with brief explanations. Let me attempt each task without a model first. Offer a hint after difficulty and invite a more precise retry.

REFERENCE

Course: Nursing and Allied Health English

Professional audience: nurses, charge nurses, physical therapists, occupational therapists, respiratory therapists, imaging staff, laboratory staff, care-team coordinators, and allied-health supervisors

Scope: Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

Lesson 01: Shift Handoffs and Clinical Prioritization

Original fictional case: In a simulated handoff, the outgoing clinician has observations and background notes, but a requested result is still pending. The receiving clinician needs a clear transfer of information.

Language workshop: Transfer information and responsibility

Communication goal: Give a handoff that the receiving person can confirm.

Language guidance: State the current situation, relevant background, open task, and receiving role. Ask for acknowledgment or repeat-back of the critical item. Sending a message and transferring responsibility are not always the same thing.

Useful frames (complete the gaps with case facts): The current status is ...; the outstanding item is ... / The record shows ... / Please confirm who has accepted ...

Vocabulary: SBAR: Situation, Background, Assessment, and Recommendation or Request: a framework for organizing a clear handoff or concern. / acuity: The severity or complexity of a patient's condition and associated care needs. / pending lab: A requested laboratory test or result that is not yet available. / watcher: A local clinical label sometimes used for a patient needing closer attention; its criteria must be clarified locally.

Speaking task: Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You are taking over the work. Ask what remains open and repeat back the critical status or responsibility. Point out one detail that would be unsafe or misleading to assume.

Writing task: Write a 70-110 word message for the person who needs to act on this case. State the purpose, preserve the relevant facts, and make the next action or unresolved question clear. Use a subject line and an appropriate opening. Do not invent a deadline, finding, or approval.

Optional second-round challenge: The intended recipient cannot take ownership. Keep the status clear and identify the need for an authorized reassignment.

Model for comparison AFTER my attempt, not a response to give me first: The result is still pending. I'll separate the current situation, relevant background, my assessment, and the request in this handoff. Please repeat back the outstanding task so we can confirm understanding.

END REFERENCE

TASK: Vocabulary in use

1. Choose four useful terms from the reference. For each, give a plain-English meaning in this field, two natural word combinations (collocations), one realistic example and a likely misuse or contrast. Keep the examples consistent with the reference and avoid jargon that does not fit this occupation.
2. Add four related terms that would be useful in a different common situation in the same field. Label these as suggested extensions, explain how they connect to the work, and flag regional or organizational variation where relevant. Do not pad the list with synonyms nobody would use at work.
3. Start a retrieval round: give one short workplace sentence with a gap and a clear clue. Ask me to supply the best term and explain my choice. Stop and wait; do not reveal the answer or a completed sentence yet. Accept another term if its meaning and collocation work.
4. After my attempt, explain one useful distinction and ask me to write my own sentence. Wait, correct a genuine meaning or usage problem, then give the next retrieval question. Work through four questions one at a time.
5. Finish with a short handoff or message task using three terms, followed by two recall questions I can save for another day. Do not pretend to schedule a reminder.

END PROMPT

Copy this prompt online or choose another lesson and support level

Test recall and transfer

START PROMPT / COPY THROUGH END PROMPT

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END REFERENCE

TASK: A short retrieval session

1. Tell me to close my notes. Plan six questions: two vocabulary-in-context items, one grammar or meaning contrast, one question about a confirmed versus unknown detail, one short spoken-style response and one new-situation transfer task. Use the supplied material as the answer reference, but do not repeat its existing checks word for word.
2. Ask only the first question, then stop and wait. Prefer a short answer over a multiple-choice question. Do not display the remaining questions, model answers or a word bank in advance.
3. After each answer, say what it gets right and explain a meaningful error if there is one. Accept alternative wording when it preserves the facts and does the communication job. If I struggle, give one clue and invite a retry before revealing a model. Only then move to the next question.
4. Keep a simple record of which items I answered independently, with a hint, or after seeing a model. This records today's practice, not a certified proficiency score. Do not judge pronunciation from typed text.
5. At the end, summarize two strengths and two useful next steps with examples from my answers. Give me a small review card containing three prompts and a separate answer section. Suggest that I revisit it tomorrow and a week later; do not claim you will remember this session or send reminders. Label the final transfer scenario as fictional.

END PROMPT

Copy this prompt online or choose another lesson and support level